Histrionic Personality Disorder (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

1. What it is

Histrionic personality disorder (HPD) is a long-standing pattern of **excessive emotionality and attention-seeking**, beginning by early adulthood and present in many settings (American Psychiatric Association [APA], 2022). It belongs to **Cluster B** (dramatic or erratic).

How common: in a large U.S. national survey, 1.84% of adults met criteria (Grant et al., 2004). Across Western community studies, rates vary widely depending on assessment method (Volkert et al., 2018).

A diagnosis with a troubled history. HPD descends from the old concept of "hysteria," long tied to beliefs about women, religion, and health. It is considered one of psychiatry's most ambiguous categories (Novais et al., 2015). Clinicians should guard against gender stereotypes: men can meet criteria too, and lively, expressive personalities are not disorders.

Counselor framing: beneath the drama is usually a fragile sense of worth that depends on being noticed. The person often feels unseen, bored, and empty when attention fades.

2. Core symptoms to watch for

DSM-5-TR criteria, paraphrased (APA, 2022); use the manual itself for formal diagnosis. A pervasive pattern, beginning by early adulthood, with **5 or more** of:

- **Uncomfortable when not the center of attention.**
- Interactions often marked by **inappropriately seductive or provocative** behavior.
- **Rapidly shifting and shallow** expression of emotions.
- Consistently uses **physical appearance** to draw attention.
- Speech that is **highly impressionistic and lacking in detail**.
- **Self-dramatization, theatricality,** and exaggerated emotional expression.
- **Suggestible,** easily influenced by others or circumstances.
- Considers relationships **more intimate than they actually are**.

Distinguish from: borderline personality disorder (self-harm, intense anger, emptiness, and identity disturbance are more prominent), narcissistic personality disorder (seeks admiration for superiority rather than any attention), dependent personality disorder, and mood episodes such as hypomania (APA, 2022).

When counselors must refer (general clinical consensus): refer for evaluation when there is depression, suicidal gestures or threats, substance use, or physical complaints that need medical review. Take every suicidal statement seriously, even if it seems attention-seeking.

3. What works: treatment and the evidence

Bottom line for class: research on HPD is very thin. Schema therapy has trial support across a group of personality disorders that included HPD; otherwise, treatment relies on cognitive and interpersonal methods.

Evidence overview (honest effect sizes):

- **Schema therapy:** in a multicenter trial of 323 patients with avoidant, dependent, obsessive-compulsive, paranoid, histrionic, or narcissistic personality disorder, schema therapy (which targets long-standing core beliefs about self and others) led to recovery more often than usual care or another psychotherapy (Bamelis et al., 2014). Few participants had HPD, so HPD-specific conclusions are uncertain.
- **No HPD-specific trials:** we found no randomized trial of any psychotherapy or medication for HPD alone (literature search, November 1, 2026).
- **Medication (referral notes):** no medication treats HPD itself. Co-occurring depression or anxiety may be treated in the usual ways.

Cognitive work with HPD: how it is used (Bamelis et al., 2014):

- **Keep sessions focused.** Gently ask for specifics ("What exactly happened next?") to counter vague, impressionistic speech.
- **Identify and challenge core beliefs:** "I am nothing unless people notice me," "If I'm not charming, I'll be ignored," "My feelings must be shown dramatically to be taken seriously," "This person really loves me" (after one meeting).
- **Build inner worth and real intimacy:** practice tolerating ordinary attention, expressing feelings directly and calmly, and building relationships slowly.

4. Religion, spirituality, and Christian integration: what the evidence shows

Disorder-specific research. We found **no study of religion or spirituality in histrionic personality disorder** and no faith-integrated treatment trial (literature search, November 1, 2026).

Related evidence. Across 49 studies, positive religious coping went with better adjustment to stress and negative religious coping with worse (Ano & Vasconcelles, 2005). Across 97 outcome studies, therapy adapted to clients' religious beliefs worked about as well as standard therapy for distress and produced added spiritual benefits (Captari et al., 2018).

Honest summary for teaching:

1. Schema-focused and cognitive approaches are the best available guide. Faith integration supports them; it does not replace them.
2. There is no research on faith and HPD. Church settings can either feed the need for attention (performance, public emotion) or offer steady acceptance that does not depend on performance.
3. Avoid labeling expressive worship or emotion as pathology; the concern is a pervasive, impairing pattern.

5. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method within cognitive, schema-focused care. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** Listen for worth tied to being seen, including in church: "If I'm not up front, I don't matter," "God only notices me when I'm on fire for Him," "People will only love me if I'm exciting."
- **Challenge (clinical).** Examine the evidence for "I am nothing without attention," and test what happens when the client serves quietly or expresses feelings calmly (Bamelis et al., 2014).
- **Replace (pastoral).** Offer Scripture that roots worth in God's unchanging gaze, not in an audience:
  - "…the LORD does not see as man does. For man sees the outward appearance, but the LORD looks at the heart." (1 Samuel 16:7)
  - "Am I now seeking the approval of men, or of God? Or am I striving to please men? If I were still pleasing men, I would not be a servant of Christ." (Galatians 1:10)
  - "I praise You, for I am fearfully and wonderfully made. Marvelous are Your works, and I know this very well." (Psalm 139:14). Worth that is given, not earned by performance.
  - "The LORD your God is among you; He is mighty to save. He will rejoice over you with gladness…" (Zephaniah 3:17). Already delighted in.
- **Faith community (clinical).** Offer warm, steady relationships with clear limits. Give service roles that build character and connection rather than constant spotlight.
- **Ministers:** respond calmly and consistently, keep boundaries clear (including around flirtation or overstated closeness), and refer when there is depression or any talk of self-harm.

6. Quick classroom takeaways

- HPD is pervasive attention-seeking and shallow, shifting emotion (5 of 8 criteria). About 1.8% of U.S. adults; beware gender bias.
- Evidence is thin; schema therapy helped a mixed group of personality disorders that included HPD.
- Replace "I am nothing unless noticed" with the truth that God sees the heart and already delights in His children.

Sources note

All 7 references were checked against Crossref, OpenAlex, and PubMed/Europe PMC records (Nov 1, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record.

References

American Psychiatric Association. (2022). *Diagnostic and statistical manual of mental disorders (5th ed., text rev.)*. https://doi.org/10.1176/appi.books.9780890425787

Ano, G. G., & Vasconcelles, E. B. (2005). Religious coping and psychological adjustment to stress: A meta-analysis. *Journal of Clinical Psychology*, *61*(4), 461–480. https://doi.org/10.1002/jclp.20049

Bamelis, L. L. M., Evers, S. M. A. A., Spinhoven, P., & Arntz, A. (2014). Results of a multicenter randomized controlled trial of the clinical effectiveness of schema therapy for personality disorders. *American Journal of Psychiatry*, *171*(3), 305–322. https://doi.org/10.1176/appi.ajp.2013.12040518

Captari, L. E., Hook, J. N., Hoyt, W. T., Davis, D. E., McElroy-Heltzel, S. E., & Worthington, E. L. (2018). Integrating clients' religion and spirituality within psychotherapy: A comprehensive meta-analysis. *Journal of Clinical Psychology*, *74*(11), 1938–1951. https://doi.org/10.1002/jclp.22681

Grant, B. F., Hasin, D. S., Stinson, F. S., Dawson, D. A., Chou, S. P., Ruan, W. J., & Pickering, R. P. (2004). Prevalence, correlates, and disability of personality disorders in the United States: Results from the National Epidemiologic Survey on Alcohol and Related Conditions. *The Journal of Clinical Psychiatry*, *65*(7), 948–958. https://doi.org/10.4088/JCP.v65n0711

Novais, F., Araújo, A., & Godinho, P. (2015). Historical roots of histrionic personality disorder. *Frontiers in Psychology*, *6*, 1463. https://doi.org/10.3389/fpsyg.2015.01463

Volkert, J., Gablonski, T.-C., & Rabung, S. (2018). Prevalence of personality disorders in the general adult population in Western countries: Systematic review and meta-analysis. *The British Journal of Psychiatry*, *213*(6), 709–715. https://doi.org/10.1192/bjp.2018.202

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